

HOSPITAL TRAINING REPORT

(LIFELINE HOSPITAL & RESEARCH CENTER, BIHAR)

A Report submitted for
Partial Fulfillment of B.Pharm VIIth Semester
Session 2025-26



Submitted by

Mukund Kumar

2310920500056

Under the supervision of

Dr. Anjali Gupta

(Professor)

Submitted to



Galgotias College of Pharmacy

(Approved by PCI and Affiliated to Dr. A.P.J. Abdul Kalam Technical University, Lucknow,
U.P.)

GALGOTIAS COLLEGE OF PHARMACY

(Approved by PCI and Affiliated to Dr. A.P.J. Abdul Kalam Technical University, Lucknow,
U.P.)



ENDORSEMENT BY THE DIRECTOR

This is to certify that Mr. Mukund Kumar, Roll No. 2310920500056 has submitted the report on “Hospital Training” subject code BP707P for partial fulfillment of B.Pharm 7th Semester, session 2026-27. The contents of this report do not form the basis for the award of any other degree to the candidate or to anybody else from this or any other university / Institutions.

Director
Dr. Vikram Sharma

GALGOTIAS COLLEGE OF PHARMACY

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U.P.)



ENDORSEMENT BY THE SUPERVISOR

This is to certify that the report on “Hospital Training” is a bonafide work under the course curriculum by Dr. A.P.J. Abdul Kalam Technical University, Lucknow for subject code BP707P submitted by Mr. Mukund Kumar, Roll No. 2310920500056 in partial fulfillment of B. Pharm 7th Semester, session 2026-27.

Name of Supervisor –
Dr. Anjali Gupta
(Professor)

GALGOTIAS COLLEGE OF PHARMACY

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U.P.)



ENDORSEMENT BY THE STUDENT

I, Mukund Kumar, Roll No. 2310920500056, hereby declare that the presented report on “Hospital Training” for subject code BP707P under the course curriculum by Dr. A.P.J. Abdul Kalam Technical University, Lucknow, for subject code BP509P has been carried out and submitted by me.

I also confirm that the report is only prepared for my academic requirement and not for any other purpose.

Mukund Kumar
Roll No – 2310920500056
B.Pharm VII Sem

INTERNSHIP CERTIFICATE

An Advanced Multi Speciality Hospital

LIFELINE HOSPITAL & RESEARCH CENTER
NEAR BARAUNI DAIRY, BARAUNI Ph. : 06279-232003, 7631009562

Web. : www.lifelinehospitalbarauni.com, e-mail : lifelinehospitalresearchcenter@gmail.com

Lt No- LLHRC/Intern/2026-2027/02 Date- 18/07/2026

Internship Certificate

This is to certify that **Mr. Mukund Kumar**, S/O Mr. Ravi Ranjan Kumar Ray a student of Galgotias College of Pharmacy, Greater Noida has completed his internship in the "Different departments including Pharmacy" at Life Line Hospital And Research Center, Barauni, under the guidance of Dr. Hemant Kumar. From 1st June 2026 to 15th July 2026.

Hospital is a 45 bedded super specialty care hospital with talented medical and paramedical professionals, the hospital provides preventive, diagnostic, therapeutic, Palliative and support services under one roof.

During this period he remained involved in his work with determination & sincerely. He is professionally sound & hard working.

I wish him every success.


Signature & Stamp

ACKNOWLEDGEMENT

The training opportunity I had with Lifeline Hospital & Research Center, Bihar, was a great chance for learning and professional development. I have learned a lot during my training duration of 45 days (1 June 2026 to 15 July 2026) and I have been fortunate in getting an opportunity of working in this hospital.

I express my deepest thanks to Dr. Hemant Kumar for providing the necessary training facility and guidance during the entire period of my training arrangement. I am also grateful for having a chance to meet so many people and professionals who guided me through this training period.

I would like to thank Dr. Vikram Sharma, Director, Galgotias College of Pharmacy, Greater Noida, for his continuous support and highest peak of encouragement for doing better in life.

I would like to acknowledge my indebtedness and deep sense of gratitude to my mentor, Dr. Anjali Gupta (Professor), for her valuable guidance and kind supervision given to me throughout the work.

I am immensely obliged to the other staff and my friends for their elevating inspiration, encouraging guidance and kind supervision in the completion of my project.

Mukund Kumar

Roll No – 2310920500056

B.Pharm 7th Sem

HOSPITAL TRAINING REPORT

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OBJECTIVES OF HOSPITAL TRAINING

- Hospital training is an observational oriented procedure in which a person is able to learn practically from their theoretical knowledge.
- Hospital training helps to study closely the ground level problems regarding their job profile.
- Hospital training provides practical knowledge to the students.
- Hospital training promotes an environment in which the student is induced to adapt themselves quickly to changed circumstances.
- Hospital training puts the students in real life situations.
- Hospital training removes the hesitation of the students regarding their working skills and personality development.

LIFELINE HOSPITAL & RESEARCH CENTER

Lifeline Hospital & Research Center was launched in 2010 in Bihar with a mission to deliver the best and affordable healthcare services to its residents. We would say that in all these years we have learned, evolved and emerged as the best hospital among all the other hospitals in Bihar.

At Lifeline Hospital & Research Center, we cover from fundamental routine checkups and outpatient services to emergency cases for which we have a trauma center and an intensive care unit. Today we can proudly say we have gathered an experience which has made us more confident and expert in handling critical medical cases. The hospital is currently 45 bedded and fully equipped with the most advanced technologies to help our patients get better soon.

At Lifeline Hospital & Research Center, we are determined to provide the best diagnostic, preventive and therapeutic services to our patients. The unwavering dedication and commitment of our specialists have made us today a multi-specialty hospital to serve our patients with the most recent and developed technology. The hospital, being a multi-specialty unit, now has the most efficient medical infrastructure and highly skilled team of doctors providing meticulous medical care in various super specialties such as Cardiology, Neurology and Neurosurgery, Urology, Plastic Surgery, Nephrology, Orthopedics, and Joint Replacement, etc. In addition to this we have advanced diagnostic and interventional radiology services and a most trusted laboratory for any kind of diagnosis.

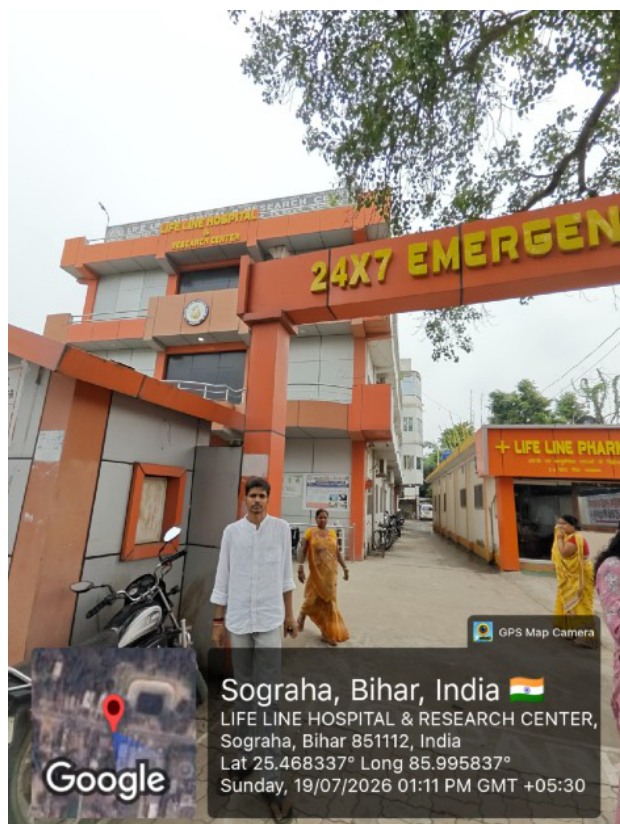


Fig 1: Hospital Entrance

FACILITIES PROVIDED AT LIFELINE HOSPITAL & RESEARCH CENTER

- 24 hrs. Emergency
- 24 hrs. Ambulance Services
- 24 hrs. Admission
- Canteen

SECTIONS IN LIFELINE HOSPITAL & RESEARCH CENTER

- General Wards
- Emergency Wards & Operation Theatre
- ICU Department
- Injection Room
- Pathology Laboratory
- Ultrasound Department
- X-Ray Department
- ECG Department
- Dressing Department
- Pharmacy

OUTPATIENT DEPARTMENT FACILITY

An outpatient department is the part of the hospital designed for the treatment of outpatients. This department provides diagnosis and care for patients that do not need to stay overnight. It is an important part of the overall running of the department. The outpatient department is usually on the ground floor of the hospital. Wheel chairs and stretchers are available for non-ambulatory patients. Close at hand are X-ray facilities, laboratories, the medical record office and a pharmacy. In the main waiting area there are a range of facilities for the patients and their families including toilets, public telephones, canteen, and a water dispenser. Some hospitals have no separate department for outpatients, so they may be treated in the same department as patients that stay overnight. At Lifeline Hospital & Research Center, there is a separate outpatient department.

Timing for OPD is 10:00 AM to 2:00 PM and 6:00 PM to 8:00 PM.



Fig 2: OPD

GENERAL WARDS

- A general ward is a large room where people who need medical treatment stay. General wards have a cubicle room having six to eight patient beds and a toilet in the wings of the ward.
- These wards are fully-equipped with a nursing station, attendant couch, and a qualified dietitian for diet advice and diet service.
- In a general ward, only those patients stay who are not suffering from any chronic disease.



Fig 3: General Wards

EMERGENCY DEPARTMENT

An Emergency Department (ED), also known as an Accident and Emergency department (A&E), Emergency Room (ER) and Casualty Department (CD).

- It is a medical treatment facility specializing in emergency medicine.
- The emergency department is usually found in a hospital or other primary care center.
- Due to the unplanned nature of patient attendance, the department must provide initial treatment for a broad spectrum of illnesses and injuries, some of which may be life threatening and require immediate attention.
- In some countries, emergency departments have become important entry points for those without other means of access to medical care.
- The emergency departments of most hospitals operate 24 hours a day, although staffing levels may vary in an attempt to reflect patient volume.



Fig 4: Emergency Department

PHARMACY DEPARTMENT

Hospital pharmacy can usually be found within the premises of the hospital.

- Hospital pharmacy usually stocks a large range of medications including more specialized and investigational medications (medicines that are being studied but have not yet been approved), more than would be feasible in the community setting.
- Hospital pharmacies typically provide medications for hospitalized patients only and are not retail establishments.
- They typically do not provide prescription service to the public.
- Some hospitals do have retail pharmacies within them, which sell over-the-counter as well as prescription medications to the public, but these are not the actual hospital pharmacy.



Fig 5: Pharmacy Department

HOSPITAL INTERNSHIP ACTIVITY

Hospital Internship Activities include:

- First Aid Treatment
- Injection
- Dressing
- Prescription
- Dispensing Procedure
- Diagnostic Report
- Patient Observation Chart

FIRST AID TREATMENT



Fig 6

FIRST AID TREATMENT

First aid is the assistance given to any person suffering a sudden illness or injury, with care provided to preserve life, prevent the condition from worsening, and/or promote recovery.

It includes initial intervention in a serious condition prior to professional medical help being available, such as performing CPR while awaiting an ambulance, as well as the complete treatment of minor conditions, such as applying a plaster to a cut.

First aid is generally performed by the layperson, with many people trained in providing basic levels of first aid, and others willing to do so from acquired knowledge.



AIMS OF FIRST AID

The key aims of first aid can be summarized in three key points, known as the “3 P’s”:

- **Preserve Life** – The overriding aim of all medical care which includes first aid is to save lives and minimize the threat of death.
- **Prevent further harm** – Also sometimes called preventing the condition from worsening or the danger of further injury; this covers both external factors such as moving a patient away from any cause of harm and applying first aid techniques to prevent worsening of the condition, such as applying pressure to stop a bleed becoming dangerous.
- **Promote recovery** – First aid also involves trying to start the recovery process from the illness or injury, and in some cases might involve completing a treatment, such as applying a plaster to a small wound.

FIRST AID SERVICES

Some people undertake specific training in order to provide first aid at public or private events, during filming or in other places where people gather. They may be designated as a first aider, or use some other title. This role may be undertaken on a voluntary basis with organizations such as the Red Cross and St. John Ambulance, or as paid employment with a medical contractor.

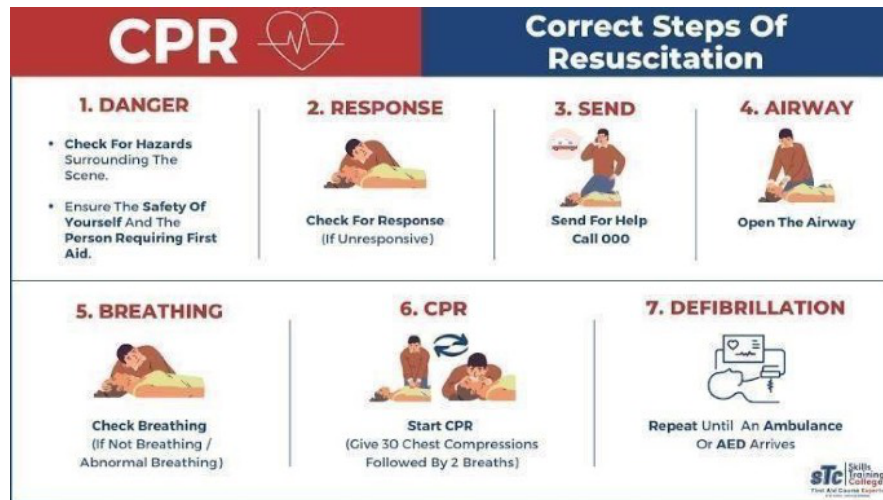
CONDITIONS THAT OFTEN REQUIRE FIRST AID

- Altitude sickness, which can begin in susceptible people at altitudes as low as 5,000 feet, can cause potentially fatal swelling of the brain or lungs.
- Anaphylaxis, a life-threatening condition in which the airway can become constricted and the patient may go into shock. The reaction can be caused by a systemic allergic reaction to allergens such as insect bites or peanuts. Anaphylaxis is initially treated with an injection of epinephrine.
- Battlefield first aid – this protocol refers to treating shrapnel, gunshot wounds, burns, bone fractures, etc., as seen either in the ‘traditional’ battlefield setting or in an area subject to damage by large-scale weaponry, such as a bomb blast.
- Bone fracture, a break in a bone initially treated by stabilizing the fracture with a splint.
- Burns, which can result in damage to tissues and loss of body fluids through the burn site.

CONDITIONS THAT OFTEN REQUIRE FIRST AID (contd.)

- Cardiac Arrest, which will lead to death unless CPR, preferably combined with an AED, is started within minutes. There is often no time to wait for the emergency services to arrive, as 92 percent of people suffering a sudden cardiac arrest die before reaching hospital, according to the American Heart Association.
- Choking, blockage of the airway which can quickly result in death due to lack of oxygen if the patient's trachea is not cleared, for example by the Heimlich Maneuver.
- Childbirth.
- Cramps in muscles due to lactic acid build-up caused either by inadequate oxygenation of the muscle or lack of water or salt.
- Gender-specific conditions, such as dysmenorrhea and testicular torsion.
- Heart attack, or inadequate blood flow to the blood vessels supplying the heart muscle.
- Hair tourniquet, a condition where a hair or other thread becomes tied around a toe or finger tightly enough to cut off blood flow.
- Heavy bleeding, treated by applying pressure (manually and later with a pressure bandage) to the wound site and elevating the limb if possible.
- Hyperglycaemia (diabetic coma) and Hypoglycaemia (insulin shock).
- Insect and animal bites and stings.
- Joint dislocation.
- Poisoning, which can occur by injection, inhalation, absorption, or ingestion.
- Seizures, or a malfunction in the electrical activity of the brain. Types include grand mal (which usually features convulsions as well as temporary respiratory abnormalities, change in skin complexion, etc.) and petit mal (which usually features twitching, rapid blinking, or fidgeting as well as altered consciousness and temporary respiratory abnormalities).
- Muscle strains and sprains, a temporary dislocation of a joint that immediately reduces automatically but may result in ligament damage.
- Stroke, a temporary loss of blood supply to the brain.
- Toothache, which can result in severe pain and loss of the tooth but is rarely life threatening, unless over time the infection spreads into the bone of the jaw and starts osteomyelitis.

CARDIOPULMONARY RESUSCITATION



- Cardiopulmonary Resuscitation (CPR) is a first aid technique to help people who suffer a “Cardiac Arrest” (their heart stops beating).
- It involves chest compressions and rescue breaths to keep the casualty alive until a defibrillator arrives.
- A defibrillator is an electrical device which can be used to help restart someone’s heart.
- CPR on its own is unlikely to restart someone’s heart; however, it will increase the chance of a defibrillator being successful.



Fig 7

INJECTIONS

- An injection is the act of putting a liquid, especially a drug, into a person's body using a needle and a syringe.
- Injection is a technique for delivering drugs by parenteral administration, that is, administration via a route other than through the digestive tract.
- Parenteral injection includes subcutaneous, intramuscular, intraperitoneal, intracardiac, intra-articular and intravenous injection.
- Injection is generally administered as a bolus, but can possibly be used for continuous drug administration as well.

Various Methods of Injections

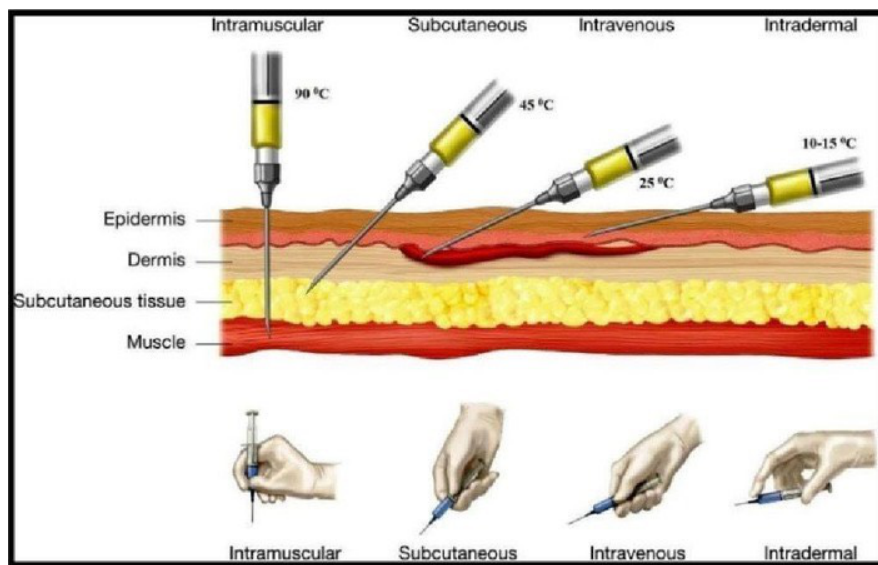


Fig 8

METHODS OF INJECTION

1. Intramuscular Injection

It involves delivering a substance deep into a muscle where it is quickly absorbed by blood vessels. The injection is often given at a 90-degree angle. Common injection sites include the deltoid, vastus lateralis and ventrogluteal muscles. Most inactivated vaccines like influenza are given by I.M. injection. Medical professionals are trained to give I.M. injections, but patients can also be trained to self-administer medications like epinephrine.

2. Subcutaneous Injection

In this, the medication is delivered to the tissue between the skin and the muscle. The injection is often given at a 45-degree angle. Absorption of the medicine is slower than that of intramuscular injections. Since the needle does not need to reach the muscle, often a bigger gauge and shorter needle is used. The usual site of administration is the fat tissue behind the arm. Insulin injection is a common type of subcutaneous injection medicine. Certain vaccines including MMR (Measles, Mumps and Rubella), Varicella (Chickenpox), and Zoster (Shingles) are given subcutaneously.

3. Intravenous Injection

It involves needle insertion directly into the vein, and the substance is directly delivered into the bloodstream. The injection is often given at a 25-degree angle. In medicine and drug use, this route is the fastest way to get the desired effect, since the medication moves immediately into blood circulation and to the rest of the body.

METHODS OF INJECTION (contd.)

4. Intradermal Injection

In this, medication is delivered directly into the dermis, the layer just below the epidermis of the skin. The injection is often given at a 5-to-15-degree angle, with the needle placed almost flat against the patient's skin. Absorption takes the longest as compared to other parenteral routes. This route is often used for sensitivity tests like tuberculin and allergy tests, and for local anesthesia tests. Common sites for this are the forearm and the lower back.

5. Intra-arterial Route

This method is used for chemotherapy in cases of malignant tumors and in angiography.

6. Intracardiac Route

Injection can be applied to the left ventricle in case of cardiac arrest.

7. Intrathecal Route

The intrathecal route involves the subarachnoid space. Injection may be applied for lumbar puncture, for spinal anesthesia and for diagnostic purposes. This technique requires special precautions.

8. Intra-articular Route

The intra-articular route involves injection into the joint cavity. Corticosteroids may be injected by this route in acute arthritis.

DRESSING



Fig 9

- A dressing is a sterile pad or compress applied to a wound to promote healing and protect the wound from further harm.
- A dressing is designed to be in direct contact with the wound, as distinguished from a bandage, which is most often used to hold a dressing in place; many modern dressings are self-adhesive.
- Dressings serve a variety of purposes depending on the type, severity and position of the wound.

Dressings are also important to help:

- Stop bleeding and start clotting so the wound can heal.
- Absorb any excess blood, plasma or other fluids.
- Wound debridement.
- Begin the healing process.

TYPES OF WOUND CARE DRESSINGS



Fig 10

What type of wound care dressing is right for my wound?

- **Hydrocolloid:** Hydrocolloid dressings are used on burns, light to moderately draining wounds, necrotic wounds, under compression wraps, pressure ulcers and venous ulcers.
- **Hydrogel:** This type of dressing is for wounds with little to no excess fluid, painful wounds, necrotic wounds, pressure ulcers, donor sites, second degree or higher burns and infected wounds.
- **Alginate:** Alginate dressings are used for moderate to high amounts of wound drainage, venous ulcers, packing wounds and pressure ulcers in stage III or IV.
- **Collagen:** A collagen dressing can be used for chronic or stalled wounds, ulcers, bed sores, transplant sites, and surgical wounds.

PRESCRIPTION

Sections of Prescription

A prescription contains handwritten instructions for the dispensing and administering of medications. It can be more than an order for drugs, as it can also include instructions for a therapist, the patient, nurse, caretaker, pharmacist or a lab technician for orders for lab tests, X-rays, and other assessments.

- **Superscription** – the heading with the date and the patient’s name, address, age, etc. Symbol Rx – the “Rx” stands for “recipe,” which in Latin means “to take.”
- **Inscription** – the information about the medication. It has the name of the ingredients and the amount needed. It includes the main ingredient, anything that helps in the action of the drug, something to modify the effects of the main drug, and the “vehicle,” which makes the medicine more pleasant to take.
- **Subscription** – tells the pharmacist how to dispense the drug. This will have instructions on compounding the drug and the amount needed.
- **Signature** – has the directions that are to be printed on the medicine label. The word “Sig” means “write on label.”

Variances in Prescription Wording

Prescriptions vary from state to state and doctor to doctor:

- Sometimes the doctor will write “dispense as written,” “do not substitute,” or “medically necessary.”
- Sometimes the age of the child is required, and often the doctor will note the condition being treated.
- Sometimes there is a label box.

DISPENSING PROCEDURE

Drug dispensing is often portrayed as merely being the process of giving a drug product to a patient. The dispensing procedure involves:

- Ensure that the prescription has the name and signature of the prescriber and the stamp of the health center.
- Ensure that the prescription is dated and has the name of the patient.
- If the prescription has not been written at a known (local) health center, the prescriber at the center should endorse it.
- Avoid dispensing without a prescription or from an unauthorized prescriber.
- Check the name of the prescribed drug against that of the container.
- Check the expiration date on the container.
- Calculate the total cost of the drug to be dispensed on the basis of the prescription where applicable.

Dispensing Prescription on Part-Payment

When a patient does not have enough money to pay for all the drugs as prescribed, the dispenser is faced with a difficult situation. The dispenser follows these steps at that time:

- If the doctor prescribed medicines for 7 days, the dispenser can dispense only 3-5 days of medicine according to the money available.
- If it is known that the medical treatment will be long, the left-over amount is written on the prescription so that it can be taken on the next occasion of dispensing.

CORRECT DRUG DISPENSING

Dispensed drugs should be appropriately labelled so that the patient can benefit optimally from the use of the drug. Expired drugs should not be dispensed.

Correct dispensing ensures that:

- The right patient is served.
- A desired dosage form of the correct drug is given.
- The prescribed dosage and quantity are given.
- The right container that maintains the potency of the drugs is used.
- The container is appropriately labelled.
- Clear instructions are delivered verbally to the patient.

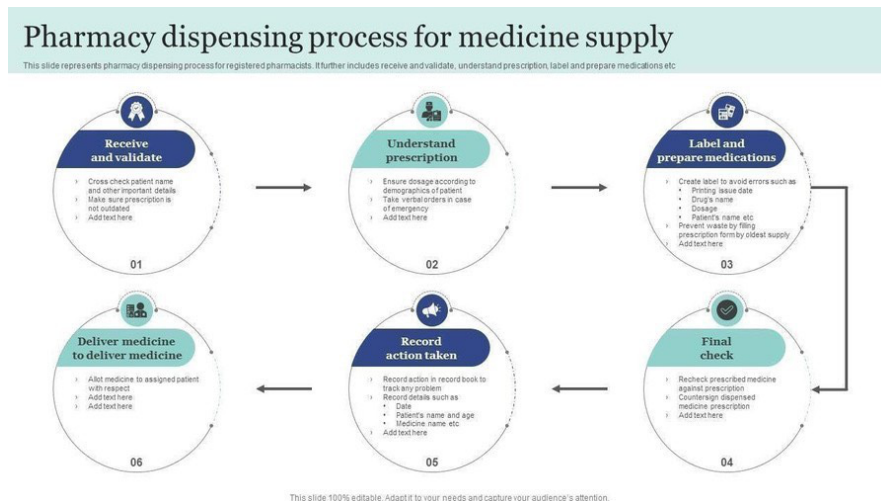


Fig 11: Pharmacy Dispensing Process for Medicine Supply

DIAGNOSTIC REPORT

- A diagnostic report is the set of information that is typically provided by a diagnostic service when investigations are complete.
- The information includes a mix of atomic results, text reports, images and codes.
- The Diagnostic Report resource is suitable for the following kinds of reports:
 - Laboratory – Clinical Chemistry, Hematology, Microbiology, etc.
 - Pathology / Histopathology related disciplines.
 - Imaging Investigations – X-Ray, CT scan, MRI, etc.
 - Other Diagnostics – Cardiology, Gastroenterology, etc.

SAMPLE DIAGNOSTIC LABORATORY REPORT



Tata 1mg Technologies Private Limited

Registered office: 2nd Floor, Plot No. B-225, Okhla Industrial Area, Phase-I, South Delhi, New Delhi-110020, Delhi
www.1mg.com/labs care@1mg.com CIN: U74140DL2015PTC279229
ISO 9001:2015



Most Trusted Brand

40 lakhs+
satisfied customers
1 crore+
lab tests booked

PO No: PO10002260234-825



Customer Name	: Mr.MUKUND KUMAR	Collected Via	: TATA 1MG OKHLA
Age/Gender	: 23Y 1M 4D /Male	Referred By	: Dr.
Lab Visit ID	: OKH3022947	Collection Date	: 08/May/2026 06:30AM
Barcode ID/Order ID	: D32290341 / 16899227	Report Date	: 08/May/2026 10:47AM
Sample Type	: Serum	Report Status	: Final Report

SEROLOGY

FEVER PACKAGE ADVANCED

Test Name	Result	Unit	Bio. Ref. Interval	Method
DENGUE FEVER NS1 ANTIGEN				
Dengue NS1 Antigen				
Result	0.2	Index	Negative (<0.9) Equivocal (0.9- <1.1) Positive (>1.1)	CLIA
Dengue NS1	Negative		Negative	

Comment:

Note: Change in method and reference interval

- Recommended test is NS1 Antigen by ELISA/CLIA in the first 5 days of fever. After 7-10 days of fever, the recommended test is Dengue virus antibodies IgG & IgM by ELISA/CLIA.
- Dengue viruses belong to the family Flaviviridae and have 4 serotypes (1-4). It is transmitted by the mosquito Aedes aegypti and Aedes albopictus and is widely distributed in Tropical and Subtropical areas of the world.
- The disease may be subclinical, self limiting, febrile or may progress to a severe form of Dengue hemorrhagic fever or Dengue shock syndrome.
- Positive: The presence of Dengue Nonstructural protein 1 (NS1) antigen is typically detectable within 1 to 2 days following infection and may persist up to 9 days following symptom onset. NS1 antigen may also be detectable during secondary dengue virus infection, but for a shorter duration of time (1-4 days following symptom onset).
- Negative: The absence of Dengue NS1 antigen suggests that the infection may not be in the acute phase. NS1 might be negative if the test is done too early (within 24-48 hours of infection) or too late (9-10 days after symptoms start). In such cases, serologic testing for IgM and IgG antibodies is recommended.
- Results should always be interpreted in conjunction with clinical presentation and exposure history.

Limitations:

- False positive results can be obtained due to cross reaction with Murray Valley and encephalitis, Japanese encephalitis, yellow fever and West Nile viruses. This occurs in less than 1% of the sample tested.

Note: The referring centre/ Lab is responsible for informing concerned Local authorities on notifiable disease.

Salmonella typhi (Typhidot) IgG & IgM

Salmonella typhi (Typhidot) IgM	Negative	NEGATIVE	Immunochromatography
Salmonella typhi (Typhidot) IgG	Negative	NEGATIVE	Immunochromatography

Comment:



This test has been performed at
TATA 1MG OKHLA
2nd Floor, B-225, Okhla Phase I, Okhla
Industrial Estate, New Delhi, Delhi 110020

Kundan

Dr. Kundan Kumar
MBBS, MD (Lab Medicine)
Consultant Laboratory Medicine
Reg No- 96030



Page 6 of 11

State of the Art Lab Facilities

Delhi • Gurgaon • Jaipur • Lucknow • Dehradun • Prayagraj • Mumbai • Pune • Ahmedabad • Bangalore • Hyderabad
Chennai • Visakhapatnam • Kolkata • Bhubaneswar • Angul • Patna • Ranchi • Guwahati

Fig 12: Serology Report

SAMPLE DIAGNOSTIC LABORATORY REPORT



Tata 1mg Technologies Private Limited

Registered office: 2nd Floor, Plot No. B-225, Okhla Industrial Area, Phase-I, South Delhi, New Delhi-110020, Delhi
www.1mg.com/labs care@1mg.com CIN: U74140DL2015PTC279229
ISO 9001:2015



Most Trusted Brand

40 lakhs+
satisfied customers
1 crore+
lab tests booked

PO No: PO10002260234-825



Customer Name	: Mr.MUKUND KUMAR	Collected Via	: TATA 1MG OKHLA
Age/Gender	: 23Y 1M 4D /Male	Referred By	: Dr.
Lab Visit ID	: OKH3022947	Collection Date	: 08/May/2026 06:30AM
Barcode ID/Order ID	: D32290342 / 16899227	Report Date	: 08/May/2026 01:55PM
Sample Type	: Whole Blood-EDTA	Report Status	: Final Report

HAEMATOLOGY

FEVER PACKAGE ADVANCED

Test Name	Result	Unit	Bio. Ref. Interval	Method
Complete Blood Count				
Hemoglobin	14.4	g/dL	13.0-17.0	Spectrophotometry (Cyanide-free)
RBC	4.57	10 ⁶ /cu.mm	4.5 - 5.5	Impedance
HCT	41.5	%	40 - 50	Calculated
MCV	90.8	fL	83 - 101	Calculated
MCH	31.4	pg	27 - 32	Calculated
MCHC	34.6	g/dL	31.5 - 34.5	Calculated
RDW-CV	13.5	%	11.5-14	Calculated
Total Leucocyte Count	3.50	10 ³ /μL	4 - 10	Impedance
Differential Leucocyte Count				
Neutrophils	40	%	40-80	DHSS/Microscopy
Lymphocytes	50	%	20-40	DHSS/Microscopy
Monocytes	08	%	2-10	DHSS/Microscopy
Eosinophils	02	%	1-6	DHSS/Microscopy
Basophils	00	%	0-2	Impedance/Microscopy
Absolute Leucocyte Count				
Absolute Neutrophil Count	1.4	10 ³ /μL	2 - 7	Calculated
Absolute Lymphocyte Count	1.75	10 ³ /μL	1-3	Calculated
Absolute Monocyte Count	0.28	10 ³ /μL	0.2 - 1	Calculated
Absolute Eosinophil Count	0.07	10 ³ /μL	0.02 - 0.5	Calculated
Absolute Basophil Count	0	10 ³ /μL	0.02-0.1	Calculated
Platelet Count	156	10 ³ /μL	150-410	Impedance/Microscopy
MPV	13.5	fL	6.5 - 12	Calculated
PDW	35.9	fL	9-17	Calculated

PLATELET COUNT rechecked at our end, suggest repeat test, if value do not correlate with clinical picture.
Platelet counts verified on the smear.

Comment:

As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.



This test has been performed at
TATA 1MG OKHLA
2nd Floor, B-225, Okhla Phase I, Okhla
Industrial Estate, New Delhi, Delhi 110020

Dr. Dhyanraj Singh
MBBS, MD(Pathology)
Consultant Pathologist
Reg No: 63325



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State of the Art Lab Facilities

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Fig 13: Haematology Report

SAMPLE DIAGNOSTIC LABORATORY REPORT



Tata 1mg Technologies Private Limited

Registered office: 2nd Floor, Plot No. B-225, Okhla Industrial Area, Phase-I, South Delhi, New Delhi-110020, Delhi
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Customer Name	: Mr.MUKUND KUMAR	Collected Via	: TATA 1MG OKHLA
Age/Gender	: 23Y 1M 4D /Male	Referred By	: Dr.
Lab Visit ID	: OKH3022947	Collection Date	: 08/May/2026 06:30AM
Barcode ID/Order ID	: D32290342 / 16899227	Report Date	: 08/May/2026 01:55PM
Sample Type	: Whole Blood-EDTA	Report Status	: Final Report

HAEMATOLOGY

FEVER PACKAGE ADVANCED

Test Name	Result	Unit	Bio. Ref. Interval	Method
DHSS : Double Hydrodynamic Sequential System Flowcytometry Calculated parameters are either derived from Impedance measure, RBC pulse measurement, RBC/platelet histograms or formula derived				

Erythrocyte Sedimentation Rate

Erythrocyte Sedimentation Rate	6	mm/hr	0-10	Capillary Photometry
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Comment:

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test.
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis, bacterial endocarditis, acute rheumatic fever, Hodgkins disease, temporal arthritis, and systemic lupus erythematosus; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection, malignancy, pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1 antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include dextran, methylidopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include aspirin, cortisone, and quinine.

Malarial Antigen (Vivax & Falciparum) Detection

Plasmodium falciparum Antigen	Negative	Negative	Immunochromatography
Plasmodium vivax Antigen	Negative	Negative	Immunochromatography

Comment:

- Four species of the Plasmodium parasites are responsible for human malaria infection - P. falciparum, P. vivax, P. ovale and P. malariae. P. falciparum and P. vivax are the most prevalent. Falciparum infection is associated with Cerebral malaria and drug resistance whereas vivax infection is associated with high rate of infectivity and relapse. Differentiation between P. falciparum and P. vivax is of utmost importance for better patient management and speedy recovery.



This test has been performed at
TATA 1MG OKHLA
2nd Floor, B-225, Okhla Phase I, Okhla
Industrial Estate, New Delhi, Delhi 110020

Dr. Dhananjay Singh
MBBS, MD (Pathology)
Consultant Pathologist
Reg No- 63325



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Fig 14: Haematology Report (contd.)

PATIENT OBSERVATION CHART

An observation and response chart is a document that allows the recording of patient observations and specifies the action to be taken in response to deterioration from the normal. The purpose of these charts is to support accurate and timely recognition of clinical deterioration and prompt action when deterioration is observed. The way in which observation charts are designed and used can contribute to both the poor recording of observations and failure to interpret them correctly. Ensuring that patients who deteriorate receive appropriate and timely care is a key safety and quality challenge.

The objective of an observation chart is to present the most important vital signs for detecting deterioration in most patients in a user-friendly manner.

- **Single parameter tool (track and trigger)** – Vital signs are compared with a simple set of criteria with predefined thresholds, with a response algorithm being activated when any criterion is met. The main vital signs are graphed so that trends can be easily ‘tracked’. There are also colour-coded zones to indicate when patient observations are likely to represent deterioration, where a response is ‘triggered’. Incorporating call criteria in observation charts is an effective way to highlight possible deterioration and assist clinicians in deciding when to ‘trigger’ a response, whether for a clinical review or a rapid response call.
- **Aggregate scoring system** – Core observations attract a weighted score. Weighted scores are assigned to physiological values and compared with predefined trigger thresholds. The main vital signs are collected and points are allocated. The points for each observation are added to give a score that helps identify patients with subtle signs of deterioration. A supporting Action Plan triggers certain actions when certain scores are reached.
- **Combination system** – Single or multiple parameter systems used in combination with aggregate weighted scoring systems.
- **Non track and trigger** – Other observation charts may include the collection of vital signs with no scoring or criteria for a response.

Hospital Name

Address

Phone Number

Website

Patient Name:

Date:

Patient Problem:

Major Illness

Problem	Start Date	Physician	Precautions

Doctors Observation on Patient:

Lorem Ipsum is simply dummy text of the printing and typesetting industry.

Lorem Ipsum has been the industry's standard dummy text ever since the 1500s, when an unknown printer took a galley of type and scrambled it to make a type specimen book.

It has survived not only five centuries, but also the leap into electronic typesetting, remaining essentially unchanged.

It was popularized in the 1960s with the release of Letterset sheets containing Lorem Ipsum passages, and more recently with desktop publishing software like Aldus PageMaker including versions of Lorem Ipsum.

[illegible]

Fig 16

CONCLUSION

During the training procedures, I have gained a lot of knowledge about the following:

- I started the project with training regarding each and every first aid procedure. It includes checking the symptoms and treating them at a small scale in first aid, and later transferring for surgical procedures.
- I gained knowledge regarding the artificial respiration process and wound dressing.
- I learned about the sites of injection, which includes knowledge of syringes and routes of injections, such as I.V., I.M., I.D., and Subcutaneous, etc.
- In prescription reading, its parts and the abbreviations used were studied by me in this project; it is truly an essential matter for a pharmacist's study.
- The dispensing procedure was also studied, which I practiced throughout the training at regular intervals.
- I also learned about the patient observation chart and how to fill it and use it.